

Wound Care Procedures

Internal policy / SOP - for demo purposes only

1. Purpose

This procedure governs the assessment, staging, treatment, and documentation of all wounds, including pressure injuries, diabetic ulcers, surgical wounds, and skin tears.

2. Initial Wound Assessment

All wounds must be assessed within 24 hours of identification by a licensed nurse. The assessment must include: anatomic location, dimensions (length x width x depth in cm), wound bed description, exudate (amount, color, consistency), periwound condition, signs of infection, and pain rating. Photographic documentation is required at admission and weekly thereafter for any wound deeper than partial thickness.

3. Pressure Injury Staging

Pressure injuries are staged per current NPIAP guidelines: Stage 1 (intact skin with non-blanchable erythema), Stage 2 (partial-thickness loss of skin), Stage 3 (full-thickness skin loss with visible subcutaneous tissue), Stage 4 (full-thickness loss with exposed bone, tendon, or muscle), Unstageable (full-thickness loss obscured by slough or eschar), and Deep Tissue Injury (intact or non-intact skin with persistent non-blanchable deep red, maroon, or purple discoloration). The stage must be reassessed weekly or with any change.

4. Treatment Selection

Wound dressing selection follows the moist-wound-healing principle. Recommended dressings by wound type: dry, low-exudate wounds -- hydrocolloid or hydrogel; moderate-exudate wounds -- foam dressing; high-exudate wounds -- alginate covered by foam. Wounds with suspected infection require culture before initiating topical antimicrobial therapy. All dressing changes must be performed using clean or sterile technique as appropriate.

5. Diabetic Foot Ulcers

Diabetic foot ulcers require offloading (specialty footwear, total-contact cast, or non-weight-bearing as ordered), strict blood-glucose control, and daily inspection of both feet. Any diabetic foot wound with signs of infection requires immediate physician notification and consideration for vascular evaluation.

6. Reassessment Schedule

Every wound must be reassessed weekly. If a wound is deteriorating (increased size, depth, exudate, or signs of infection) the physician must be notified and the treatment plan revised. Wounds that have not progressed after two weeks of appropriate treatment require wound-care nurse consultation.

7. Infection Surveillance

Signs of wound infection include increased pain, erythema extending more than 2 cm beyond the wound edge, increased or purulent exudate, malodor, induration, or warmth. Systemic signs (fever, elevated white blood cell count) raise concern for cellulitis or osteomyelitis and require immediate physician notification.

8. Documentation

Wound assessments and treatments must be documented in the wound-care flow sheet at every dressing change. Weekly summary documentation must include measurements, photographs, treatment plan, and trajectory (improving, stable, or deteriorating).